

ADRABETADEX

PROVIDER NET COST RECOVERY MODEL

CONCEPT REVIEW BRIEF

Gross WAC does not show whether a treatment center is financially whole after acquisition cost, reimbursement, operating burden, claim friction, and payment timing.

WAC / ADMIN	ANNUAL ADMINS	ANNUAL GROSS WAC	340B PROXY	INITIAL PART B REF.
\$ \$39,000	# 26	^ \$1.014M	H \$29,991	✓ \$40,170

1. WHY THE MODEL IS BENEFICIAL

LAUNCH QUESTION	WHAT THE CALCULATOR MAKES VISIBLE	BENEFIT
Can a provider afford to start treatment?	Drug acquisition, reimbursement, and timing exposure by scenario	Identifies where uptake is workable, thin, or blocked
Does WAC overstate provider economics?	WAC, acquisition basis, reimbursement, and net recovery as separate layers	Prevents list price from being treated as provider margin
Which sites should be prioritized?	COE/HOPD, non-340B, office, Medicaid, and white-bagging pathways	Supports practical launch sequencing
What support changes the result?	Support required to move a negative case to neutral	Ties support design to a specific provider gap

3. SCENARIO SIGNAL

SCENARIO	ACQUISITION	REIMBURSEMENT BASIS	DRAFT SIGNAL
340B COE / HOPD	\$29,991	\$40,170 benchmark logic	WORKABLE
Non-340B HOPD	\$39,000	\$40,170 benchmark logic	THIN TO NEGATIVE
Commercial buy-and-bill + 5% support	\$37,050	Medical benchmark	POTENTIALLY NEUTRAL
Commercial buy-and-bill + 10% support	\$35,100	Medical benchmark	MORE CONSISTENTLY NEUTRAL
Commercial white-bagging	\$0 provider acquisition	Administration only	NO DRUG SPREAD
Medicaid non-340B	State-specific	PAD or MCO logic	HIGH VARIANCE

4. COMPARATOR USE AND REVIEW BOUNDARIES

SPINRAZA COMPARATOR AS INTRATHECAL ANALOGUE

SPINRAZA POINT	PUBLIC EVIDENCE	WHY IT IS USED
Intrathecal route	Confirmed in U.S. label [S1]	Shared administration context
Billing unit	J2326, 0.1 mg unit [S2]	Drug-to-dose translation discipline
Procedure sequence	CPT 96450 commonly billed same encounter [S3]	Drug and procedure economics are separate
WAC/list context	Recent public list-price references reported by Reuters [S4]	Market reference only
ASP reporting status	ASP file status does not determine coverage [S6]	Explicit data-availability constraint

REVIEW BOUNDARIES

APPROPRIATE STATEMENT	BOUNDARY
Compares acquisition, reimbursement, burden, and payment timing	Does not guarantee coverage or payment

This is a reimbursement execution tool built from the provider perspective. It converts payer rules and channel mechanics into center-level economics.

2. PROVIDER ECONOMICS BRIDGE

LAYER	MODEL OUTPUT
Gross exposure	Per-dose and annual exposure (WAC x dosing cadence)
Acquisition basis	Provider cost at risk (WAC, 340B proxy, support, or no provider acquisition)
Reimbursement basis	Expected allowed payment (payer/site benchmark, contract, state method, or interim-code logic)
Operating burden	Margin erosion (labor, handling, billing/admin, financing, denial rework)
Net recovery	Workable, thin, negative, or state-dependent scenario signal

5. ADRABETADEX VS INTRATHECAL ANALOGUE FRAMING

ECONOMIC LAYER	ADRABETADEX	SPINRAZA ANALOGUE	WHAT THIS DEMONSTRATES
Gross price input	\$39,000 per administration	Public WAC/list references available [S4]	Gross price is only the first input
Billing structure	Product-specific code pending	J2326, 0.1 mg unit [S2]	Coding structure affects claim translation
Payment benchmark	Initial WAC-based reference, payer-specific thereafter	ASP/payment visibility may vary [S6]	Payment logic must be modeled separately
Acquisition basis	WAC, 340B proxy, support, or channel path	Site/channel dependent	Provider cost is not automatically WAC
Result	Scenario-calculated	Scenario-calculated if inputs are available	Provider economics are modeled, not assumed

6. SOURCE NOTES

- SCENARIO BASED
- PROVIDER FOCUSED
- RISK & SUPPORT AWARE
- LAUNCH READY

S3 CMS/Noridian billing guidance for Spinraza